

Pathogen of the Week

Weekly briefing for policymakers · WHO · Africa CDC · ECDC · US CDC · UKHSA · PHAC · open-source signals

Week 20 · Mon 11 – Sun 17 May 2026

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THIS WEEK AT A GLANCE

A new Public Health Emergency of International Concern has been declared. The briefing is restructured around it.

Bottom line: WHO declared the Ebola Bundibugyo outbreak in DRC's Ituri Province and Uganda a PHEIC on 16 May 2026 — 10 lab-confirmed and ~246 suspected cases, ~80 suspected deaths, with cross-border spread to Kampala via two travellers. This is only the third recorded outbreak of Bundibugyo ebolavirus (after Uganda 2007 and DRC 2012). There are no approved BVD-specific therapeutics or vaccines; historical BVD case-fatality has been ~25–35%. The Andes hantavirus MV Hondius cluster has grown to 12 cases (9 confirmed, 2 probable, 1 inconclusive) and docks at Rotterdam on 18 May, but global risk remains assessed as low. Mpox clade Ib is the smoldering background story with continued community transmission across five European countries. H5N1 has been moved off the active list for this week.

ACUTE EVENT · LEAD STORY · PHEIC

LEAD

Ebola Bundibugyo

Bundibugyo ebolavirus (BVD) — DRC Ituri outbreak with cross-border spread to Uganda

DRC Ituri Province (Bunia, Rwampara, Mongbwalu) · imported case cluster in Kampala, Uganda

10

LAB-CONFIRMED CASES

~80

SUSPECTED DEATHS

~25–35%

HISTORICAL BVD CFR

Severity

9.5

Novelty

8.5

Attention

9.5

Policy

9.5

Pandemic

4.5

Composite: 8.3 / 10

WHY IT MATTERS

WHO declared this outbreak a Public Health Emergency of International Concern on 16 May 2026. It is only the third recorded outbreak of Bundibugyo ebolavirus — a rare ebolavirus species for which no approved therapeutics or vaccines exist (the Ervebo vaccine and approved monoclonal antibodies target Zaire ebolavirus). The two prior BVD outbreaks were confined within national borders: Uganda's Bundibugyo District in 2007 (149 cases, 37 deaths, ~25% CFR) and DRC's Isiro health zone in 2012 (38 confirmed cases, 13 deaths, ~34% CFR). This is the first known emergence of BVD in DRC's Ituri Province, and the first time the species has spread internationally during an active outbreak.

FOR POLICYMAKERS

- Activate cross-border surveillance and exit screening between DRC and Uganda, Rwanda, South Sudan and CAR; brief national EOCs on BVD-specific clinical case definitions.
- Stand up isolation and ETC capacity in Bunia and Kampala; pre-position PPE and rapid PCR panels capable of distinguishing BVD from Zaire ebolavirus.
- Coordinate with WHO R&D Blueprint and IFFIm on emergency-use access to investigational BVD vaccines and mAbs; existing Zaire-targeted countermeasures cannot be assumed cross-protective.

ACUTE EVENT · ONGOING

Andes hantavirus

Orthohantavirus andesense (ANDV) — MV Hondius cluster

Atlantic crossing · docks Rotterdam 18 May 2026 · contacts in 19 countries

12

CASES (9 CONFIRMED, 2 PROBABLE, 1 INCONCLUSIVE)

3

DEATHS

~25%

CFR (CURRENT)

Severity

8.5

Novelty

7.0

Attention

7.5

Policy

7.5

Pandemic

2.5

Composite: 6.6 / 10

WHY IT MATTERS

The cluster has grown to 12 cases (9 confirmed) since first flagged. The vessel docks at Rotterdam on 18 May 2026 with passengers and crew of 19 nationalities, triggering a coordinated Dutch / ECDC / WHO disembarkation and contact-tracing operation. Sequencing continues to point to limited human-to-human transmission — Andes is the only hantavirus species with documented person-to-person spread. WHO still assesses global public-health risk as low.

FOR POLICYMAKERS

- Coordinate cross-border contact tracing for repatriated passengers (incubation window up to ~6 weeks).
- Support Dutch port-of-entry disembarkation plan: medical triage, onboard isolation of any symptomatic contacts, environmental decontamination of cabins.
- Brief cruise lines and travel-health authorities on rodent-exclusion and onboard isolation protocols.

SMOLDERING · PHEIC

Mpox (clade Ib)

Monkeypox virus, clade Ib

DRC, Madagascar, Uganda, Burundi, Kenya · community transmission in 5 European countries

907

CONFIRMED (AFRICA, RECENT PERIOD)

7

DEATHS (RECENT PERIOD)

74

CLADE I · EU/EEA (FEB–MAR)

Severity

5.5

Novelty

4.0

Attention

5.0

Policy

7.0

Pandemic

4.5

Composite: 5.2 / 10

WHY IT MATTERS

The overall African trend is declining since May 2025, but sustained community transmission of clade Ib continues in Austria, Belgium, Portugal, Spain and the UK, mostly in sexual networks of MSM. A WHO situation report (#65) and an ECDC weekly threats note both updated this month. A novel clade Ib/IIb recombinant has been flagged in WHO DON 2026-DON595 — worth monitoring. The DRC overlap with the new Ebola Bundibugyo outbreak compounds the operational burden on national health authorities.

FOR POLICYMAKERS

- Sustain targeted JYNNEOS / MVA-BN vaccine availability for at-risk groups; supply tightening in some markets.
- Maintain non-stigmatizing messaging through community organizations; avoid travel restrictions.
- Fund continued genomic surveillance — clade I/II recombinants are emerging.

Full sources, interactive radar, expanded details → <https://ivetyorda.github.io/pathogen-of-the-week/>

Week 20 · 2026 · auto-regenerated every Sunday